



Name	Mrs.SUKHLEEN KOUR	Sample Received	12-11-2024 12:22 PM	Sample Collected At	LIFE SECURE CLINICAL LABORATORY
Age/Gender	20 Y / Female	Sample Processed	12-11-2024 12:22 PM	Referred By	LIFE SECURE CLINICAL LABORATORY
UID	HLD1211202442558	Approved Date	12-11-2024 04:05 PM		

LABORATORY REPORT

*HBA1C, GLYCATED HEMOGLOBIN

RESULT : 4.5 %

Interpretation

HbA1c result is suggestive of non diabetic adults (≥ 18 years)/ well controlled Diabetes in a known Diabetic.

Interpretation as per American Diabetes Association (ADA) Guidelines

Reference Group	Non diabetic adults ≥ 18 years	At risk (Prediabetes)	Diagnosing Diabetes	Therapeutic goals for glycemic control
HbA1c in %	4.0-5.6	5.7-6.4	≥ 6.5	< 7.0

Note: Presence of Hemoglobin variants and/or conditions that affect red cell turnover must be considered, particularly when the HbA1C result does not correlate with the patient's blood glucose levels

FACTORS THAT INTERFERE WITH HbA1C MEASUREMENT	FACTORS THAT AFFECT INTERPRETATION OF HBA1C RESULTS
Hemoglobin variants,elevated fetal hemoglobin (HbF) and chemically modified derivatives of e.g., carbamylated Hb in patients with renal failure) can affect the accuracy of HbA1c measurements	Any condition that shortens erythrocyte survival or decreases mean erythrocyte age (e.g., recovery from acute blood loss hemolytic anemia, HbSS, HbCC, and HbSC) will falsely lower HbA1c test results.regardless of the assay method used.Iron deficiency anemia is associated with higher HbA1c

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BIO CHEMISTRY REPORT

Investigations	Values	Unit	Bio-logical Reference
*GLUCOSE FASTING	81.4	mg/dl	70.0 - 110.0
Method: Spectrophotometry, Hexokinase			
*CALCIUM	9.24	mg/dl	8.7 - 10.4
Method: Arsenazo III			

LIVER FUNCTION SCREENING

*BILIRUBIN TOTAL	0.62	mg/dL	0.3 - 1.1
Method: Vanadate oxidation			
*BILIRUBIN CONJUGATED (DIRECT)	0.15	mg/dL	0.0 - 0.3
Method: Vanadate oxidation			
*BILIRUBIN (INDIRECT)	0.47	mg/dL	0.0 - 0.9
Method: Calculated			
*ASPARTATE AMINOTRANSFERASE (AST/SGOT)	29.2	U/L	0.0 - 34.0
Method: UV without Pyrodoxal Po4			
*ALANINE AMINOTRANSFERASE (ALT/SGPT)	14.3	U/L	10.0 - 49.0
Method: UV without Pyrodoxal Po4			
*ALKALINE PHOSPHATASE (ALP)	77.1	U/L	45.0 - 129.0
Method: Modified IFCC			
*PROTEIN, TOTAL	7.3	g/dl	5.7 - 8.2
Method: Biuret			
*ALBUMIN	4.77	g/dl	3.2 - 4.8
Method: BCG Dye Binding			

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BIO CHEMISTRY REPORT

Investigations	Values	Unit	Bio-logical Reference
GLOBULIN. Method: Calculated	2.53	g/dl	3.0 - 4.2
*A/G RATIO Method: Calculated	1.89	g/dl	1.2 - 2.0

RENAL FUNCTION SCREENING

*UREA Method: Enzymatic colorimetric	12.58	mg/dL	10.0 - 50.0
*CREATININE Method: Picrate Method	0.51	mg/dL	0.4 - 1.1
*URIC ACID Method: UV Method	3.98	mg/dL	3.0 - 6.0
*BLOOD UREA NITROGEN Method: Calculated	5.88	mg/dL	5.0 - 23.0
BUN/CREATININE RATIO. Method: Calculated	11.53	mg/dL	10.0 - 25.0
UREA/CREATININE RATIO. Method: Calculated	24.67	mg/dL	20.0 - 50.0

ELECTROLYTES SERUM.

*SODIUM Method: Ion Selective Electrode	137	mEq/L	135.0 - 145.0
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BIO CHEMISTRY REPORT

Investigations	Values	Unit	Bio-logical Reference
*POTASSIUM	3.66	mEq/L	3.5 - 5.5
Method: Ion Selective Electrode			
*CHLORIDE	105	mEq/L	96.0 - 110.0
Method: Ion Selective Electrode			

LIPID PROFILE

*TOTAL CHOLESTEROL	128.1	mg/dl	0.0 - 200.0
Method: ChodPap			
*TRIGLYCERIDES	78	mg/dl	0.0 - 150.0
Method: Enzymatic			
HDL CHOLESTEROL.	49.5	mg/dl	40.0 - 60.0
Method: Direct			
*LDL CHOLESTEROL	63.0	mg/dl	0.0 - 130.0
Method: Calculated			
VLDL CHOLESTEROL.	15.6	mg/dl	6.0 - 30.0
Method: Calculated			
CHOL / HDL RATIO.	2.59	Ratio	4.0 - 6.0
Method: Calculated			
LDL / HDL RATIO.	1.27	Ratio	0.5 - 3.5
Method: Calculated			
TRIGLYCERIDES/HDL RATIO.	1.58	Ratio	0.1 - 3.1
Method: Calculated			

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BIO CHEMISTRY REPORT

Investigations	Values	Unit	Bio-logical Reference
NON-HDL CHOLESTEROL.	78.6	mg/dl	0.0 - 130.0
Method: Calculated			
TOTAL LIPIDS.	334.2	mg/dl	0.0 - 700.0
Method: Calculated			

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BIO CHEMISTRY REPORT

***GFR, ESTIMATED**

RESULT: 183.49 mL/min/1.7m2

AGE IN YEARS	20 - 29	30 - 39	40 - 49	50 - 59	60 - 69	>=70
GFR IN mL/min/1.7m2	116	107	99	93	85	75

Normal GFR	Mild decrease in GFR	Moderate decrease in GFR	Sever decrease in GFR	Kidney Failure
>90	60 - 89	30 - 59	15 - 29	<15

Note:

1. National Kidney Disease Education program recommends the use of MDRD equation to estimate or predict GFR in adults (>=20 years) with Chronic Kidney Disease (CKD)

2. MDRD equation is most accurate for GFR <=60 mL/min/1.7m2

Critical value of Creatinine established as per laboratory policy: Adult : > 5.0

Such critical value if obtained needs urgent medical attention

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HAEMATOLOGY REPORT

Investigations	Values	Unit	Bio-logical Reference
*HEMOGLOBIN	12.84	g/dl	12.0 - 15.0
Method: Cyan-methemoglobin			
*PCV	39.0	%	36.0 - 46.0
Method: Mathematical Calculatio			
*RBC COUNT	3.94	million/ μ l	4.5 - 5.5
Method: Light scatter			
MCV.	98.9	fL	83.0 - 101.0
Method: Mathematical Calculation			
MCH.	32.6	pg	27.0 - 32.0
Method: Mathematical Calculation			
MCHC.	32.9	g/dl	31.0 - 34.5
Method: Mathematical Calculation			
R.D.W.	15.1	%	11.5 - 14.5
Method: Measured, RBC Histogram			
*PLATELET COUNT	153	1000 cells/ μ l	150.0 - 410.0
Method: Light scatter			
MPV.	12.72	fL	6.8 - 10.9
Method: Measured Platelet Histogram			
*TOTAL LEUCOCYTE COUNT (TLC)	5.27	1000cells/ μ L	4.0 - 10.0
Method: Peroxidase/Basophil/Lobularity			
*NEUTROPHILS	40.0	%	40.0 - 75.0

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HAEMATOLOGY REPORT

Investigations	Values	Unit	Bio-logical Reference
Method: Peroxidase (Flowcytometry)			
*ABSOLUTE NEUTROPHIL COUNT (ANC)	2.11	1000/ μ l	2.0 - 7.0
Method: Peroxidase (Flowcytometry)			
*LYMPHOCYTES	47.0	%	20.0 - 45.0
Method: Peroxidase (Flowcytometry)			
*ABSOLUTE LYMPHOCYTE COUNT(ALC)	2.48	1000/ μ l	1.0 - 3.0
Method: Peroxidase (Flowcytometry)			
*MONOCYTES	10.0	%	1.0 - 10.0
Method: Peroxidase (Flowcytometry)			
*ABSOLUTE MONOCYTE COUNT(AMC)	0.53	1000/ μ l	0.2 - 1.0
Method: Peroxidase (Flowcytometry)			
*EOSINOPHILS	3.0	%	1.0 - 6.0
Method: Peroxidase (Flowcytometry)			
*ABSOLUTE EOSINOPHIL COUNT(AEC)	0.16	1000/ μ l	0.02 - 0.5
Method: Peroxidase (Flowcytometry)			
*BASOPHILS	0.0	%	0.0 - 1.0
Method: Basophil/Lobularity(Flowcytometry)			
*ABSOLUTE BASOPHIL COUNT (ABC)	0.0	1000/ μ l	0.0 - 0.1
Method: Basophil/Lobularity(Flowcytometry)			

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